SECOND EDITION

You're Not Overreacting

An ER doctor's guide to what's normal, what's not, and when to get care for your baby



Dr. Oliver Morris, DO
Emergency Physician · Dad of Four

When Every Second Counts

Keep this page easy to find. If something feels life-threatening, don't read. Call.

Call 911 if your child:

- Isn't breathing, or has a breathing pause of 20 seconds or more (put 911 on speaker and start CPR)
- Is choking and can't cry, cough or breathe (start back blows, Chapter 25)
- Is struggling to breathe, or has blue or gray lips, tongue or face
- Has a first-ever seizure, or one lasting 5 minutes or longer
- Won't wake up, or is limp and unresponsive
- Has a severe injury or heavy bleeding
- Has lip or tongue swelling, trouble breathing, or repeated vomiting after a food, medicine or sting (give epinephrine first if you have it)

1-800-222-1222

Poison Control, 24/7, free and confidential. Call right away for any medicine, chemical or plant your child may have swallowed, even if they seem fine. Don't make them vomit. Trouble breathing, a seizure, or won't wake up? Call 911.

Go to the emergency department now if your child:

- Is under 2 months old with a temperature of 100.4°F (38°C) or higher (don't give fever medicine first)
- Is very hard to wake, floppy, or can't be comforted
- Is working hard to breathe: ribs pulling in, grunting, nostrils flaring, or too breathless to feed
- Stopped breathing, turned blue or gray, or went limp, even if they seem fine now
- Has green vomit, blood in vomit, or more than a streak of blood in stool
- Shows dehydration: no pee in 6 to 8 hours, no tears, very dry mouth, or can't keep sips down
- Has purple or red spots that don't fade when pressed, with fever or looking sick
- Swallowed a button battery, a magnet or water beads, even if they seem fine
- Had a head injury, then passed out, vomited more than once, or is unusually sleepy

Call your pediatrician today if:

- Your baby is 2 to 3 months old with a temperature of 100.4°F (38°C) or higher (call right away, even at night)
- Fever lasts more than 24 hours under age 2, or more than 3 days at age 2 and older
- Your baby is feeding much less, or has fewer wet diapers than usual
- A symptom keeps getting worse instead of better
- Your child seems "off" and you can't say why
- You're worried. That's reason enough.

You don't need to be certain, and you don't need permission. If you're torn, get your child seen. At your limit with crying? Put your baby down safely and step away. If you might hurt yourself or your baby, call 911. For US families. General information, not medical advice.
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Important Information

You're Not Overreacting: An ER Doctor's Guide to What's Normal, What's Not, and When to Get Care for Your Baby.
Second Edition.

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Medical disclaimer

This book is for general education. It's written to help parents understand common concerns in babies and young children, recognize warning signs, and feel more confident deciding when to get care.

It is not medical advice. It can't diagnose or treat your child, and it doesn't replace an exam by a doctor, nurse practitioner or physician assistant who can see your child in person. Reading it does not create a doctor-patient relationship.

Every child is different. If your child looks unwell, if symptoms are getting worse, or if something just feels wrong to you, get medical care right away. If you think your child is having a medical emergency, call 911.

Medication information in this book is general. Always check the label on your specific product, dose by your child's current weight, and confirm with your pediatrician or pharmacist, especially for babies under 2 years old.

Guidance in pediatrics changes as new research comes out. This edition reflects recommendations current as of October 2026. When your own pediatrician's advice differs from this book, follow your pediatrician. They know your child.

This book is written for families in the United States. Emergency numbers, Poison Control and medicine strengths are different in other countries. If you live outside the US, use your local emergency number and follow the dosing on your own product's label or your pharmacist's advice.

The views in this book are the author's own and don't represent any hospital, employer or medical group. The author has worked to make this information accurate, but medicine changes and mistakes can happen. It is provided as is, without warranties, and the author is not responsible for any loss or harm from its use.

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Welcome

If you're holding this book, you've probably had that moment. It's 2 a.m., your baby is making a sound you've never heard before, and you're standing over the crib with your phone in one hand trying to decide: is this nothing, or is this something?

I've been on both sides of that moment. As an emergency physician, I've spent close to a decade with families who come through the ER doors scared and, very often, apologizing. "I'm sorry, I'm probably overreacting." As a dad of four, including twins, I've stood over that crib too.

Here's what I tell every one of those parents: you're not overreacting. The instinct that something isn't right is one of the best tools we have in emergency medicine. Many serious illnesses get caught early because a parent noticed something small. And when the answer turns out to be "your baby is okay," that's not a wasted trip. That's a good outcome.

This book is the conversation I have at the bedside, written down so you have it before you need it.

How this book is organized

There are 25 short chapters, grouped into eight parts. Every chapter opens with an **At a Glance** page, built so you can make a decision quickly when you're tired and worried. The full chapter follows for when you have a few minutes to understand what's going on.

The At a Glance pages use the same colors throughout:

- **Green, usually normal.** Things that look or sound alarming but are typically part of being a baby.
- **Blue, what helps at home.** Practical things you can do right now.
- **Amber, call your pediatrician.** Usually not an emergency, but call now, not tomorrow. If the office is closed, use the after-hours line. Your child may need to be seen today or the next morning. A baby under 3 months with a fever always needs a call right away, day or night.
- **Coral, go to the ER now.** Don't wait for morning. Drive if your child is stable, or call 911 if you're not sure.
- **Red, call 911.** Life-threatening signs. Call first, then read.

If your child fits more than one color, follow the most urgent one.

Three questions to ask every time

No matter what the symptom is, the same three questions will steer you in the right direction. They're a parent's version of what ER doctors check in the first few seconds with every child:

1. **How does my child look and act?** Alert, interested, consolable? Or unusually sleepy, floppy, or not themselves?
2. **How hard is my child working to breathe?** Easy and comfortable, even if noisy? Or fast, effortful, with skin pulling in at the ribs?
3. **What color is my child, and are they keeping fluids in?** Normal color and wet diapers? Or pale, gray or blue, with fewer wet diapers?

If all three answers are reassuring, you usually have time to watch, comfort and call your pediatrician if things change. If any one of them worries you, get help. A few situations need care even when your child looks fine: any fever in a baby under 2 months, a swallowed button battery, magnet or medicine, passing out after a head injury, and any episode where your baby stopped breathing, turned blue or went limp.

A few things before you start

Read the chapters on fever, breathing and safe sleep now, before your baby is sick. Save Poison Control (1-800-222-1222) in your phone today. Fill out the "My Child's Health Info" page at the back. And if you haven't taken an infant CPR class, sign up for one. It's two hours that could matter more than anything else in this book.

Most of all, use this guide to feel more confident, never as a reason to wait. If something feels wrong, trust that feeling.

Dr. Oliver Morris, DO

Contents

PART 1 · START HERE

1 Trust Your Instincts 8

2 Where to Get Care 12

PART 2 · FEVER

3 Fever 17

PART 3 · BREATHING

4 Normal Breathing Patterns 23

5 Noisy Breathing 28

6 Stuffy Nose and Congestion 32

7 Cough and Croup 36

8 Wheezing, Bronchiolitis and RSV 41

9 Fast or Hard Breathing 46

PART 4 · FEEDING AND TUMMY

10 Reflux and Spitting Up 51

11 Gas and Colic 55

12 Constipation and Stool Patterns 59

13 Vomiting, Diarrhea and Dehydration 64

PART 5 · SKIN

14 Diaper Rash 70

15 Common Rashes 74

16 Skin Color Changes and Jaundice 80

PART 6 · NEWBORN CARE AND SLEEP

17 Umbilical Cord Care 85

18 Normal Newborn Quirks 89

19 Sleep and Safe Sleep 94

PART 7 · CRYING AND SCARY MOMENTS

20 Inconsolable Crying 100

21 Seizures 105

22 Scary Episodes That Stop on Their Own 110

PART 8 · INJURIES AND EMERGENCIES

23 Head Bumps and Falls 116

24 Poisoning and Swallowed Medicines 121

25 Choking and Swallowed or Stuck Objects 126

RESOURCES

A Final Word 131

Normal Numbers at a Glance 132

Fever Medicine Dosing Chart 134

Home Sick-Day Kit 136

My Child's Health Info 137

Symptom and Medicine Log 138

Sources 139

About the Author 146

PART 2

Fever

What the thermometer can and can't tell you.

3 Fever

CHAPTER 03 · AT A GLANCE

Fever

The number matters less than you think



USUALLY NORMAL

- Over 3 months: fever with a cold
- Over 3 months: fussy and tired, perks up after medicine
- Faster breathing and heart rate while hot
- Eating less, but still drinking and peeing

WHAT HELPS AT HOME

- Light clothing and plenty of fluids
- Fever medicine dosed by weight, for comfort only
- Use the syringe that comes in the box
- Write down every dose and the time

CALL YOUR PEDIATRICIAN

- Fever at 2 to 3 months (call right away, even at night)
- Fever lasting more than 24 hours under age 2
- Fever lasting more than 3 days at age 2 and older
- Fever above 104°F (40°C) more than once
- Still looks sick after the fever comes down

GO TO THE ER NOW

- Any fever in a baby under 2 months
- Floppy, very hard to wake, or can't be comforted
- Working hard to breathe
- Stiff neck, bulging soft spot, or a rash that won't fade
- Dehydration: dry mouth, no tears, no pee in 6 to 8 hours
- Fever with sickle cell, cancer care or a weak immune system

Call 911 Blue or gray lips or face · Unresponsive or not breathing normally · A first-ever seizure, or one lasting 5 minutes or longer

THE BOTTOM LINE

Under 2 months, any fever means the ER. After that, how your child acts matters more than the number.

CHAPTER 3 · FEVER

Fever

The thermometer beeps, you read the number, and your stomach drops. 103°F. Is that dangerous? Should you be in the car right now?

Take a breath. For most children past the newborn stage, fever is not the enemy. It's the body turning up the heat to fight off an infection, usually a simple virus. What tells you whether your child is okay is their age, how they look and act, and how long the fever lasts. The number on the screen comes a distant fourth.

What counts as a fever

A fever is a temperature of **100.4°F (38°C) or higher**. Anything below that isn't a fever, even if your child feels warm to you. In babies under 3 months, trust only a rectal reading. A normal forehead or armpit number doesn't rule out a fever at this age.

How you measure matters, especially in young babies:

Method	Best for	Notes
Rectal	Under 3 months (and the most accurate method at any age)	The method to use in young babies
Forehead (temporal artery)	Any age	Next most accurate; follow the instructions exactly; under 3 months, check rectally before you decide
Ear	6 months and older	Not reliable in younger babies
Armpit	Any age, for screening	Least accurate; can miss a fever
Mouth	Age 4 and up	Wait 30 minutes after hot or cold drinks

TRY THIS:

How to take a rectal temperature. Clean a digital thermometer and put a little petroleum jelly on the tip. Lay your baby on their back with legs lifted toward the chest (like a diaper change), or tummy-down across your lap. Turn the thermometer on and gently slide the tip in about 1/2 inch for babies under 6 months, or up to 1 inch for older babies. Stop if you feel resistance. Hold it in place, keeping a hand on your baby, until it beeps. Label that thermometer "rectal" and don't use it in the mouth.

Age changes everything

Under 2 months: Any fever of 100.4°F (38°C) or higher means the ER, right away, even if your baby looks fine. Young babies can have serious infections (in the blood, urine, or the lining of the brain) while looking only a little off. Don't give fever medicine first unless a doctor tells you to. It can hide the fever and won't change the plan.

2 to 3 months: Call your pediatrician right away, day or night. Don't wait for morning. Many babies this age need to be seen that same day or night. If you can't reach anyone quickly, go to the ER.

Low temperatures matter too. In a baby under 3 months, a rectal temperature of 96.8°F (36°C) or lower can be a sign of serious infection. Call your pediatrician right away, day or night. If your baby also seems floppy, very sleepy, or isn't feeding well, go to the ER.

Older than 3 months: Now we judge mostly by behavior. A child who is drinking, peeing, breathing comfortably, and perks up when the fever comes down can usually be cared for at home.

Vaccination status matters too. Babies who haven't had their first rounds of shots, or who are behind, are less protected against some serious bacterial infections. Tell the doctor if your child is behind on vaccines, was born early, or spent time in the NICU, because each can change how closely they want to check a fever.

> **FROM THE ER:**
> A toddler at 103°F who's grumpy but sipping juice and watching cartoons worries me far less than a toddler at 100.8°F who's limp and won't look at me. I treat the child, not the thermometer.

Fever myths

> **MYTH:**
> A high fever will cause brain damage. **Fact:** Fevers from ordinary infections don't damage the brain. Fevers from infection rarely go above 106°F (41.1°C), and when they do, the cause of the fever is what we worry about, not the heat itself. That's why a temperature of 106°F (41.1°C) or higher needs the ER right away. Heatstroke, like a child left in a hot car, is a different and true emergency. Call 911.

Two more you'll hear often:

- **"It's just teething."** Teething can nudge the temperature up slightly, but research shows it does not cause a real fever. A temperature of 100.4°F (38°C) or higher is probably something else.
- **"You have to get the fever back to normal."** You don't. Medicine is for comfort. If your child is resting or playing happily with a fever, you don't need to treat it.

Medicines: using them safely

Acetaminophen (Tylenol) and **ibuprofen** (Motrin, Advil) both work well. The rules:

- **Acetaminophen under 12 weeks:** only if a clinician tells you to.
- **Ibuprofen:** only for babies 6 months and older. Skip it if your child is vomiting a lot or seems dehydrated.
- **Never give aspirin** to children. It's linked to a rare but dangerous liver and brain condition called Reye's syndrome.
- **Dose by weight, not age.** Weight is more accurate. See the dosing chart at the back of this book.
- **Use the syringe or cup that comes in the box.** Kitchen spoons aren't accurate, and devices from different products don't match.
- **Write down every dose and the time.** When you're exhausted, it's easy to forget, and double doses happen. Cough and cold medicines aren't safe for young children (see Chapter 6), and many combination products for older kids already contain acetaminophen, so check the label before giving more.

Alternating acetaminophen and ibuprofen is common, but it raises the chance of dosing mistakes and isn't needed for most fevers. Do it only if your child's doctor advises it, and track every dose carefully.

Comfort and fluids

- Dress your child in light clothing and use a light blanket. Don't bundle a feverish child.
- Offer fluids often. Babies under 6 months need breast milk or formula, not plain water. Older kids can have water, an electrolyte drink, or ice pops.
- Let them rest. Sleep is fine; you don't need to wake a child to give fever medicine.
- **Don't** use cold baths, ice, or sponging with cool water. They often cause shivering and make kids feel worse, without helping more than medicine does. **Never** rub alcohol on the skin. It can be absorbed or inhaled and is dangerous.

When the fever hangs on, or climbs

Most viral fevers last 2 to 3 days. Call your pediatrician if a fever lasts **more than 24 hours in a child under 2**, or **more than 3 days** at age 2 and older. A fever lasting **5 days or more** needs a doctor's visit. One rare cause is **Kawasaki disease**, an illness that inflames blood vessels and can affect the heart. Watch for red eyes, a rash, red cracked lips, swollen hands or feet, or a swollen gland in the neck. Early treatment protects the heart.

A fever that **repeatedly goes above 104°F (40°C)** at any age deserves a call. So does a single reading over 104°F that doesn't come down with medicine.

Some children between 6 months and 5 years have a seizure with a fever, often on the first day of an illness. These febrile seizures are frightening and usually harmless, but every one needs a doctor. Call 911 for a first-ever seizure, even if it stops, or for any seizure that lasts 5 minutes or longer. If your child has had febrile seizures before, and this one was short and they're back to normal, call your pediatrician right away. See Chapter 21 (Seizures) for what to do during one.

When to get help

CALL YOUR PEDIATRICIAN RIGHT AWAY, EVEN AT NIGHT, IF:

- Your baby is 2 to 3 months old with a fever of 100.4°F (38°C) or higher. If you can't reach anyone quickly, go to the ER. - Your baby is under 3 months and has a rectal temperature of 96.8°F (36°C) or lower - Your child has had febrile seizures before, had another short one, and is back to normal - Your child has a fever and a heart condition or another serious medical condition

CALL YOUR PEDIATRICIAN TODAY IF:

- The fever lasts more than 24 hours (under 2) or more than 3 days (age 2 and older) - The fever goes above 104°F (40°C) more than once, or your child seems sick after it comes down - Your child has ear pain, a bad sore throat, or burning with peeing

GO TO THE ER NOW IF:

- Your baby is under 2 months old with any fever - The temperature is 106°F (41.1°C) or higher - Your child has sickle cell disease, is getting cancer treatment, or takes medicine that weakens the immune system, unless their specialist gave you a different fever plan - Your child is floppy, very hard to wake, or can't be comforted - Breathing is fast and hard, with ribs pulling in - There's a stiff neck, a bulging soft spot in a baby, a severe headache, or a rash that doesn't fade when pressed - Your child isn't drinking, has had no wet diaper in 6 to 8 hours, or has no tears

RED FLAG:

A fever in a baby under 2 months old is an emergency, even if the baby seems fine.

CALL 911 IF:

- Lips or face look blue or gray, or your child isn't responding - It's a first-ever seizure, or a seizure lasts 5 minutes or longer

Your eyes are the best fever tool you have. Look at your child first, then the thermometer.

Fever Medicine Dosing Chart

Always dose by weight, not age. Check the concentration on your bottle, use the syringe or cup that came in the box, and write down every dose with the time. If your child's weight isn't on this chart, or you're unsure, call your pediatrician or pharmacist.

Acetaminophen (Tylenol and store brands)

Liquid strength: **160 mg per 5 mL** (US infant and children's liquids are the same strength). **US products only:** outside the US, including Canada, infant drops can be much stronger, so use your country's chart. Give every 4 to 6 hours as needed, no more than 5 doses in 24 hours. **Under 12 weeks old: only if your doctor tells you to.** The AAP also advises checking with your pediatrician before giving it to any child under 2.

Weight	Liquid 160 mg/5 mL	Chewable 160 mg
6 to 11 lb	1.25 mL, only if your doctor tells you to	–
12 to 17 lb	2.5 mL	–
18 to 23 lb	3.75 mL	–
24 to 35 lb	5 mL	1 tablet
36 to 47 lb	7.5 mL	1½ tablets
48 to 59 lb	10 mL	2 tablets
60 to 71 lb	12.5 mL	2½ tablets
72 to 95 lb	15 mL	3 tablets

Ibuprofen (Motrin, Advil and store brands)

Only for babies 6 months and older. Give every 6 to 8 hours as needed, no more than 4 doses in 24 hours. Give with food or milk. Avoid if your child is dehydrated or vomiting a lot.

Weight	Infant drops 50 mg/1.25 mL	Children's liquid 100 mg/5 mL	Chewable 100 mg
12 to 17 lb	1.25 mL	2.5 mL	–
18 to 23 lb	1.875 mL	4 mL	–
24 to 35 lb	2.5 mL	5 mL	1 tablet
36 to 47 lb	3.75 mL	7.5 mL	1½ tablets
48 to 59 lb	5 mL	10 mL	2 tablets
60 to 71 lb	–	12.5 mL	2½ tablets
72 to 95 lb	–	15 mL	3 tablets

Watch the strengths. Infant ibuprofen drops are more concentrated than children's ibuprofen liquid. Mixing them up is one of the most common dosing errors I see.

Safety rules

- Never give aspirin to children or teens.
- The chewable columns are for 160 mg acetaminophen and 100 mg ibuprofen tablets only. Check the mg on your package.
- Don't give cough and cold medicines to children under 6 unless your pediatrician says to. If you use any combination product, check it for hidden acetaminophen or ibuprofen before giving more.
- Don't wake a sleeping child just to give fever medicine.
- If a single medicine isn't keeping your child comfortable, ask your pediatrician before alternating the two. If you do alternate, write down every dose.
- Treat discomfort, not the number. A child over 3 months who is playing and drinking doesn't need medicine just because the thermometer says 101. Any fever in a baby under 3 months still needs a call right away, or the ER under 2 months.

For US products only. General information, not medical advice. Confirm doses with your pediatrician or pharmacist.

About the Author

Oliver Morris, DO, is an emergency physician, board-certified in emergency medicine by the American Board of Emergency Medicine, with nearly a decade of experience caring for infants, children and families in the emergency department. He practices at a teaching hospital, where he trains emergency medicine residents and mentors physicians in training.

In the ER, he has sat with worried parents who arrive scared, uncertain, and apologizing for coming in. Again and again, he found himself explaining the same things: what's normal, what's not, and when to trust your gut. Those conversations so often ended with a parent saying, "I wish someone had told me this sooner."

That's why this book exists. Too many parents leave the pediatrician's office, urgent care, or the ER without the guidance they were looking for. Dr. Morris wanted to put that same bedside conversation, honest, reassuring and practical, into the hands of every parent who has ever searched a symptom at 2 a.m. and wondered if they were overreacting.

He's also a dad of four, including a set of twins. He knows firsthand that parenting is equal parts love and worry, and he wrote this book as much from that side of the bed as from the doctor's.